

D454

Adult Health III

Armored Drill

8 concepts · 12 items · 4 transfer items

What this is. Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

Source. Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

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How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

Items

1.

A 67-year-old man who retired last year after a career as a high-school band director is two days out from a large anterior MI. His BP is 82/50, HR 118, skin cool and clammy, crackles in both lung bases, jugular veins distended, and PAWP 24 mm Hg. His daughter is at the bedside asking about visiting hours. Which type of shock best fits these findings?

- A. Hypovolemic shock, because any hypotensive patient should be assumed dry until proven otherwise.
- B. Cardiogenic shock.
- C. Septic (distributive) shock.
- D. Hypovolemic shock.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

2. [TRANSFER — reversed stem / different variable]

Which hemodynamic finding would the nurse expect early in septic shock?

- A. Bradycardia with warm, dry skin below an injury level.
- B. Elevated PAWP with crackles.
- C. Low systemic vascular resistance.
- D. High systemic vascular resistance.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

3.

An 80-kg woman admitted from a long-term care facility known for its garden program has a urinary source of infection, temperature 39.2 °C, BP 84/46, and lactate 4.4 mmol/L. Her chart notes a lactate of 1.1 at a clinic visit six months ago. Blood cultures have been drawn and antibiotics are hanging. The order reads “crystalloid 30 mL/kg.” What total volume should the nurse expect to give?

- A. Hold fluids until today’s lactate is compared with the one from six months ago.
- B. 2,400 mL
- C. 30 mL
- D. 1,000 mL, a standard single bolus.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

4. [TRANSFER — reversed stem / different variable]

After her fluid bolus, a septic patient's BP is 90/42. What is her mean arterial pressure, and what does it mean?

- A.** 74 mm Hg; goal met.
- B.** 66 mm Hg; goal met.
- C.** 48 mm Hg; the patient needs another 30 mL/kg bolus.
- D.** 58 mm Hg; below the 65 goal, so a vasopressor is likely next.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

5.

A 58-year-old landscaper on a ventilator after aspiration pneumonia has bilateral infiltrates not explained by heart failure. On PEEP 10 and FiO₂ 0.50, his PaO₂ is 75 mm Hg. The family brought in a photo board of his garden projects. How should the nurse interpret his ARDS severity?

- A.** Mild ARDS, since any ratio above 100 is mild.
- B.** Moderate ARDS; P/F ratio 150.
- C.** Severe ARDS; P/F ratio 75.
- D.** Not ARDS, because ARDS can only be graded on 100% oxygen.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

6.

A man who was riding a rented electric scooter is admitted after a fall with a small subdural hematoma. At 0800 he was alert and oriented. At 1100 he is restless, keeps pulling at his gown, and answers questions slowly; BP 132/80, HR 84, pupils equal and reactive. He has a history of seasonal allergies. Which finding is the nurse's earliest sign of rising ICP?

- A.** The new restlessness and slowed responses.
- B.** His vital signs, because they have not yet shown Cushing's triad.
- C.** Nasal congestion from his allergies.
- D.** A fixed, dilated pupil if it appears.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

7. [TRANSFER — reversed stem / different variable]

A patient with a traumatic brain injury has a MAP of 78 mm Hg and an ICP of 22 mm Hg. What is the cerebral perfusion pressure?

- A.** 22 mm Hg
- B.** 100 mm Hg
- C.** 78 mm Hg
- D.** 56 mm Hg, below the usual goal of 60–70.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

8.

A college sophomore with type 1 diabetes arrives with glucose 520 mg/dL, pH 7.18, positive ketones, and Kussmaul breathing. Her roommate says she has been studying for finals and skipping meals. Isotonic fluids are running. Her potassium is 3.0 mEq/L, and an insulin infusion is ordered. What should the nurse do before starting the insulin?

- A.** Give sodium bicarbonate to correct the pH of 7.18.
- B.** Start the insulin now, since high glucose is the main problem.
- C.** Give 50% dextrose IV push.
- D.** Hold the insulin, replace potassium as ordered, and confirm with the provider.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

9.

A patient with acute kidney injury who coaches youth soccer on weekends has a potassium of 6.8 mEq/L and new peaked T waves. His urine output has been 15 mL/hour for four hours. Which medication should the nurse expect to give first?

- A.** IV regular insulin with dextrose.
- B.** IV potassium chloride.
- C.** IV calcium gluconate.
- D.** An oral potassium binder such as patiomer.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

10. [TRANSFER — reversed stem / different variable]

Which intervention actually removes potassium from the body in hyperkalemia?

- A.** Hemodialysis.
- B.** Nebulized albuterol, which always clears the excess potassium.
- C.** IV calcium gluconate.
- D.** Insulin with dextrose.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	____	

11.

A woman with cirrhosis who runs a small bakery was admitted confused with asterixis. On lactulose she now has three soft stools a day and is oriented again. She prefers to be called by her middle name. She asks the nurse to stop the lactulose because of the stools. What is the best response?

- A.** Explain that 2-3 soft stools a day means the medicine is removing ammonia, and it should continue.
- B.** Hold the lactulose, since loose stools mean it should be stopped.
- C.** Ask the provider for a sedative to calm her if confusion returns.
- D.** Tell her the stools prove her ammonia is normal, so no further monitoring is needed.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	____	

12.

A 61-year-old man with advanced heart failure who collects vintage radios is being evaluated for a heart transplant. He is short of breath and anxious most days and asks for help feeling better while he keeps going to transplant appointments. His insurance is through a union plan. Which service best fits his request?

- A.** Palliative care, but only after he signs a DNR order.
- B.** Wait until the transplant is denied, then offer palliative care.
- C.** A palliative care consult.
- D.** Hospice enrollment.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	____	

STOP

Answers and dissection follow. Finish every item and every strip first.

Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

1. Answer B Concept 1

A 67-year-old man ~~who retired last year after a career as a high school band director~~ is two days out from a large anterior MI. His BP is 82/50, HR 118, skin cool and clammy, crackles in both lung bases, jugular veins distended, and PAWP 24 mm Hg. ~~His daughter is at the bedside asking about visiting hours.~~ Which type of shock best fits these findings?

Target word: shock classification **Decides:** Crackles, JVD, and high PAWP with hypotension after MI point to pump failure.

A	Absolute language	A blanket rule would lead to fluid boluses that worsen pulmonary congestion here.
B	Correct	Pump failure backs blood up into the lungs (crackles, high PAWP, JVD) while output falls.
C	Adjacent category	Distributive shock is driven by vasodilation and low SVR, usually with warm skin early; no infection source here.
D	Confusion set	Shares cool skin and tachycardia, but hypovolemia gives low PAWP and flat neck veins.

2. Answer C Concept 1 • transfer

Which hemodynamic finding would the nurse expect early in septic shock?

Target word: expected profile for a named shock type — reverse direction **Decides:** Septic shock is distributive: widespread vasodilation.

A	Adjacent category	Describes neurogenic shock, another distributive type with a different cause.
B	Confusion set	That is the cardiogenic pattern.
C	Correct	Vasodilation drops SVR; that is the defining feature of distributive shock.
D	Inverted operation	High SVR is the compensatory pattern in hypovolemic and cardiogenic shock.

3. Answer B Concept 2

An 80-kg woman admitted from a long-term care facility ~~known for its garden program~~ has a urinary source of infection, temperature 39.2 °C, BP 84/46, and lactate 4.4 mmol/L. ~~Her chart notes a lactate of 1.1 at a clinic visit six months ago.~~ Blood cultures have been drawn and antibiotics are hanging. The order reads “crystalloid 30 mL/kg.” What total volume should the nurse expect to give?

Target word: weight-based bolus volume **Decides:** 30 mL × 80 kg.

A	Camouflage	The old lactate does not change today’s bundle; hypotension and lactate ≥ 4 trigger fluids now.
B	Correct	$30 \text{ mL/kg} \times 80 \text{ kg} = 2,400 \text{ mL}$.
C	Dropped element	Leaves out the weight multiplier.
D	Adjacent category	A routine bolus size is not the weight-based sepsis dose ordered here.

4. Answer D Concept 2 • transfer

After her fluid bolus, a septic patient's BP is 90/42. What is her mean arterial pressure, and what does it mean?

Target word: solve for MAP and compare to goal **Decides:** $MAP = (SBP + 2 \times DBP) / 3 = (90 + 84) / 3$.

A	Inverted operation	Doubles the systolic instead of the diastolic: $(180 + 42) / 3 = 74$.
B	Dropped element	Simple average of 90 and 42 ignores that diastole takes about two-thirds of the cycle.
C	Confusion set	48 is the pulse pressure, not the MAP.
D	Correct	$(90 + 84) / 3 = 58$, under the $MAP \geq 65$ target after adequate fluids.

5. Answer B Concept 3

~~A 58-year-old landscaper~~ on a ventilator after aspiration pneumonia has bilateral infiltrates not explained by heart failure. On PEEP 10 and FiO₂ 0.50, his PaO₂ is 75 mm Hg. ~~The family brought in a photo board of his garden projects.~~ How should the nurse interpret his ARDS severity?

Target word: ARDS grade **Decides:** $PaO_2/FiO_2 = 75 / 0.50 = 150$.

A	Adjacent category	Mild is 201–300; 101–200 is moderate.
B	Correct	$75 / 0.50 = 150$, which falls in the 101–200 band.
C	Wrong stem number	Uses the PaO ₂ by itself as the ratio.
D	Absolute language	The ratio is calculated at whatever FiO ₂ is in use, with PEEP ≥ 5 .

6. Answer A Concept 4

~~A man who was riding a rented electric scooter~~ is admitted after a fall with a small subdural hematoma. At 0800 he was alert and oriented. At 1100 he is restless, keeps pulling at his gown, and answers questions slowly; BP 132/80, HR 84, pupils equal and reactive. ~~He has a history of seasonal allergies.~~ Which finding is the nurse's earliest sign of rising ICP?

Target word: earliest ICP sign **Decides:** A new change in level of consciousness.

A	Correct	Declining LOC is the earliest and most sensitive sign of rising ICP.
B	Inverted operation	Cushing's triad is late; normal vitals do not rule out rising ICP.
C	Camouflage	The allergy history is irrelevant to ICP.
D	Adjacent category	Pupil changes signal later herniation risk, not the earliest change.

7. Answer D Concept 4 • transfer

A patient with a traumatic brain injury has a MAP of 78 mm Hg and an ICP of 22 mm Hg. What is the cerebral perfusion pressure?

Target word: solve for CPP **Decides:** $CPP = MAP - ICP$.

A	Wrong stem number	That is the ICP given in the stem.
B	Inverted operation	Adds ICP to MAP instead of subtracting.
C	Dropped element	Uses MAP alone and ignores the pressure opposing flow.
D	Correct	$78 - 22 = 56$.

8. Answer D Concept 5

A college sophomore with type 1 diabetes arrives with glucose 520 mg/dL, pH 7.18, positive ketones, and Kussmaul breathing. ~~Her roommate says she has been studying for finals and skipping meals.~~ Isotonic fluids are running. Her potassium is 3.0 mEq/L, and an insulin infusion is ordered. What should the nurse do before starting the insulin?

Target word: action before insulin **Decides:** Potassium 3.0 is below 3.3.

A	Adjacent category	Bicarbonate is reserved for very severe acidosis (pH below about 6.9); fluids and insulin fix the acidosis.
B	Dropped element	Ignores the low potassium that insulin will push lower.
C	Confusion set	That treats hypoglycemia; dextrose is added in DKA only once glucose falls to about 200–250.
D	Correct	Insulin drives K ⁺ into cells; starting it with K ⁺ under 3.3 risks dangerous hypokalemia and arrhythmia.

9. Answer C Concept 6

A patient with acute kidney injury ~~who coaches youth soccer on weekends~~ has a potassium of 6.8 mEq/L and new peaked T waves. His urine output has been 15 mL/hour for four hours. Which medication should the nurse expect to give first?

Target word: first drug in hyperkalemia with ECG changes **Decides:** ECG changes mean the heart must be protected first.

A	Confusion set	Shifts K ⁺ into cells and is given soon after, but calcium comes first for the heart.
B	Inverted operation	Adds potassium to a patient who already has too much.
C	Correct	Calcium stabilizes the cardiac membrane within minutes; it buys time but does not lower K ⁺ .
D	Adjacent category	Binders remove potassium but act over hours; not first with ECG changes.

10. Answer A Concept 6 · transfer

Which intervention actually removes potassium from the body in hyperkalemia?

Target word: remove vs. shift vs. stabilize **Decides:** Only excretion or dialysis takes K⁺ out.

A	Correct	Dialysis pulls potassium out of the blood.
B	Absolute language	Albuterol only shifts K ⁺ into cells, and its effect wears off.
C	Confusion set	Protects the heart but leaves serum K ⁺ unchanged.
D	Adjacent category	Moves K ⁺ into cells temporarily; total body potassium is unchanged.

11. Answer A Concept 7

A woman with cirrhosis ~~who runs a small bakery~~ was admitted confused with asterixis. On lactulose she now has three soft stools a day and is oriented again. ~~She prefers to be called by her middle name.~~ She asks the nurse to stop the lactulose because of the stools. What is the best response?

Target word: lactulose goal **Decides:** 2–3 soft stools a day is the therapeutic target and her confusion has cleared.

A	Correct	The stools are the intended effect; stopping risks a return of encephalopathy.
B	Inverted operation	Treats the goal as a side effect to be eliminated.
C	Adjacent category	Sedatives are avoided because the liver cannot clear them and they worsen encephalopathy.
D	Absolute language	Mental status and labs are still monitored; improvement is not a guarantee.

12. Answer C Concept 8

A 61-year-old man with advanced heart failure ~~who collects vintage radios~~ is being evaluated for a heart transplant. He is short of breath and anxious most days and asks for help feeling better while he keeps going to transplant appointments. ~~His insurance is through a union plan.~~ Which service best fits his request?

Target word: palliative vs. hospice **Decides:** He wants symptom relief while still pursuing treatment.

A	Absolute language	Palliative care does not require a DNR.
B	Inverted operation	Reverses the model: palliative care starts with symptoms, not after treatment ends.
C	Correct	Palliative care manages symptoms alongside active treatment at any stage.
D	Confusion set	Hospice is for a prognosis of about six months or less once curative treatment stops.

Triage grid

Write each item number in the box it belongs to.

	Right	Wrong
Sure	Owned.	Concept inversion — top priority. A wrong mental model; it survives rewording.
Unsure / guess	Recognition, not mastery. Re-drill with the transfer.	Not learned yet.

Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

Trap	What it does	In this drill	Your misses
Dropped element	Leaves out a required term or step.	4	
Wrong stem number	A figure copied from the prompt that is not the answer.	2	
Adjacent category	The right idea in the wrong bucket.	9	
Inverted operation	Direction flipped: add/subtract, cause/effect, higher/lower.	7	
Absolute language	Always, never, guarantee — where judgment applies.	5	
Confusion set	Every option is real; the test is exact role-matching.	7	
Camouflage	Answers a stem fact that was never the question.	2	

Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

#	Concept	Rule	Trap	Tell
1	Classifying shock by the failing component	Hypovolemic shock = low volume (low preload, high SVR); cardiogenic = pump failure (high preload/PAWP, high SVR, crackles, JVD); distributive (septic, anaphylactic, neurogenic) = vasodilation with low SVR.	Treating every hypotensive patient as hypovolemic and pushing large fluid boluses.	Crackles and JVD, or warm flushed skin, alongside hypotension.
2	Sepsis bundle and perfusion targets	In suspected sepsis draw lactate and blood cultures, then give broad-spectrum antibiotics within the first hour; give 30 mL/kg crystalloid for hypotension or lactate ≥ 4 mmol/L and target MAP ≥ 65 mm Hg, adding a vasopressor if fluids do not get there.	Delaying antibiotics until culture results return, or judging perfusion by systolic pressure alone.	Infection source plus fever, tachycardia, hypotension, or high lactate.
3	ARDS severity and refractory hypoxemia	ARDS is graded by PaO ₂ /FiO ₂ on PEEP ≥ 5 : mild 201–300, moderate 101–200, severe ≤ 100 ; care uses low tidal volume (about 6 mL/kg predicted body weight), PEEP, and prone positioning when severe.	Grading severity by PaO ₂ alone or expecting more oxygen alone to fix the hypoxemia.	Hypoxemia that persists despite higher FiO ₂ , with bilateral infiltrates.
4	Increased intracranial pressure	The earliest sign of rising ICP is a change in level of consciousness; Cushing's triad (widening pulse pressure, bradycardia, irregular respirations) is late. CPP = MAP – ICP, goal about 60–70 mm Hg.	Waiting for Cushing's triad or blown pupils before acting.	New restlessness, confusion, or drowsiness after a head injury or stroke.
5	DKA treatment sequence	DKA care begins with isotonic IV fluids; IV insulin starts only once potassium is at least 3.3 mEq/L, and dextrose is added when glucose falls to about 200–250 mg/dL while insulin continues until the anion gap closes.	Starting insulin immediately regardless of potassium, or stopping insulin as soon as glucose normalizes.	High glucose, low pH, ketones, Kussmaul breathing, and a potassium value in the stem.
6	Hyperkalemia emergency order	For hyperkalemia with ECG changes: IV calcium first to stabilize the heart, then shift K ⁺ into cells (insulin with dextrose, albuterol), then remove it (binders, loop diuretics, dialysis).	Thinking calcium lowers potassium, or starting with a slow-acting binder.	Peaked T waves and K ⁺ above 6 in a patient with kidney injury.

#	Concept	Rule	Trap	Tell
7	Hepatic encephalopathy and lactulose	In cirrhosis, rising ammonia causes confusion and asterixis; lactulose is titrated to about 2-3 soft stools a day to trap and excrete ammonia.	Holding lactulose because the patient has loose stools, or giving sedatives for the agitation.	Cirrhosis with new confusion or a flapping hand tremor.
8	Palliative care vs. hospice	Palliative care relieves symptoms at any stage and can run alongside curative or life-prolonging treatment; hospice is palliative care for a prognosis of about six months or less once curative treatment stops.	Believing palliative care means giving up treatment or requires a DNR order.	A seriously ill patient still pursuing treatment who wants symptom relief.

Course scope (public description): Adult Health III prepares students to provide safe, equitable, high quality complex medical surgical nursing care for diverse adult populations across the lifespan in various health care settings. This includes health promotion and management of both acute and chronic conditions. This course explores how social determinants of health impact health risk and outcome. This course will focus on caring for patients with potentially life-threatening alterations of the respiratory, cardiovascular, endocrine, and neurologic, renal, hepatic systems, end of life care (palliative), shock and transplants. Clinical judgment and problem solving are emphasized in the assessment of critically ill patients and prioritizing patient 's needs and nursing interventions. A variety of populations and settings are used in the experiential learning components of this course. Co-requisites: D455